

Discharge Summary

Patient Information

Patient Name: John Doe
Medical Record Number: 123456789
Date Admitted: July 1, 2050
Date Discharged: July 3, 2050
Attending Physician: Dr. Will Teachwell
Resident Physician: Dr. Bea Goodoc

Diagnoses

- Left-sided systolic congestive heart failure (CHF)
- Type 2 Diabetes
- Stage 2 Hypertension
- Osteoarthritis

History of Present Illness

Mr. Doe is a 72-year-old gentleman with a history of CHF who presented with a 3-day history of gradually worsening lower extremity edema, weight gain, and shortness of breath. Today while trying to mow his lawn he had to stop multiple times to catch his breath and developed some chest tightness leading him to come to the emergency room. He denies any other decrease in exercise tolerance or chest pain leading up to this event. He reports good understanding and adherence to his medications, but does report that when his daughter was visiting town 4 days prior to admission that he and his wife took her out for a special dinner and he indulged in high-sodium foods.

Clinical Course & Lab Results

1. **Congestive Heart Failure:** On admission (July 1, 2050), he had 3+ lower extremity pitting edema, elevated jugular venous pressure, and a CXR consistent with volume overload. He was diuresed and was able to quickly wean off supplemental oxygen. Exam revealed resolution of his edema at discharge. Discharge weight is 202 lbs. An echocardiogram performed previously on April 1, 2050, showed an **EF of 35%**.
2. **Chest Pain:** Ruled out for an MI with serial EKGs and enzymes. His EKGs remained unchanged from previous and his enzyme curve remained normal. Chest pain was attributed to his hypoxia from his CHF.
3. **Type 2 Diabetes:** Labs drawn on July 1, 2050, showed an **HgbA1C of 7.3**, which is good control for him at this time. No changes were made to his diabetic regimen.

4. **Hypertension:** Initially elevated at the time of admission, his BP came down nicely with diuresis and was **142/82** once euvoletic. In keeping with guidelines, we increased his lisinopril and metoprolol for optimal BP control. He responded well and BP was **126/72** at discharge.

Discharge Medications

Medications continued with dose changes:

- Metoprolol 50 mg PO BID
- Lisinopril 40 mg PO qd

Medications continued unchanged:

- ASA 81 mg PO qd
- Furosemide 40 mg PO BID
- Spironolactone 50 mg PO qd
- Metformin 1000 mg PO BID
- Glipizide 10 mg PO BID
- Tylenol 325 mg PO q 4 hours as needed for pain

Follow-up Instructions

Primary Care: Friday, July 7, 2050 at 11:30 AM

Cardiology: Thursday, July 27, 2050 at 2:30 PM